CHAPTER 191

AN ACT concerning staff working with persons with developmental disabilities or traumatic brain injury and supplementing Titles 30 and 45 of the Revised Statutes.

BE IT ENACTED *by the Senate and General Assembly of the State of New Jersey:*

C.30:6D-5.1 Short title.

1. This act shall be known and may be cited as "Danielle's Law."

C.30:6D-5.2 Definitions relative to staff working with persons with developmental disabilities, traumatic brain injury.

2. As used in this act:

"Commissioner" means the Commissioner of Human Services.

"Department" means the Department of Human Services.

"Facility for persons with developmental disabilities" means a facility for persons with developmental disabilities as defined in section 3 of P.L.1977, c.82 (C.30:6D-3).

"Facility for persons with traumatic brain injury" means a facility for persons with traumatic brain injury that is operated by, or under contract with, the department.

"Life-threatening emergency" means a situation in which a prudent person could reasonably believe that immediate intervention is necessary to protect the life of a person receiving services at a facility for persons with developmental disabilities or a facility for persons with traumatic brain injury or from a public or private agency, or to protect the lives of other persons at the facility or agency, from an immediate threat or actual occurrence of a potentially fatal injury, impairment to bodily functions or dysfunction of a bodily organ or part.

"Public or private agency" means an entity under contract with, licensed by or working in collaboration with the department to provide services for persons with developmental disabilities or traumatic brain injury.

C.30:6D-5.3 Responsibilities of staff at facility for persons with developmental disabilities, traumatic brain injury.

3. a. A member of the staff at a facility for persons with developmental disabilities or a facility for persons with traumatic brain injury or a member of the staff at a public or private agency, who in either case works directly with persons with developmental disabilities or traumatic brain injury, shall be required to call the 911 emergency telephone service for assistance in the event of a life-threatening emergency at the facility or the public or private agency, and to report that call to the department, in accordance with policies and procedures established by regulation of the commissioner. The facility or the public or private agency, as applicable, and the department shall maintain a record of such calls under the policy to be established pursuant to this section.

b. The department shall ensure that appropriate training is provided to each member of the staff at a facility for persons with developmental disabilities or a facility for persons with traumatic brain injury or member of the staff at a public or private agency, who in either case works directly with persons with developmental disabilities or traumatic brain injury, to effectuate the purposes of subsection a. of this section.

C.30:6D-5.4 Violations, penalties.

4. A member of the staff at a facility for persons with developmental disabilities or a facility for persons with traumatic brain injury or a member of the staff at a public or private agency who violates the provisions of section 3 of this act shall be liable to a civil penalty of $5,000 for the first offense, $10,000 for the second offense, and $25,000 for the third and each subsequent offense, to be sued for and collected in a summary proceeding by the commissioner pursuant to the "Penalty Enforcement Law of 1999," P.L.1999, c.274 (C.2A:58-10 et seq.).

C.30:6D-5.5 Record of violations.

5. The department shall maintain a record of violations of the provisions of section 3 of this act, which shall be included in the criteria that the department considers in making a decision on whether to renew the license of a facility or whether to renew a contract with a public or private agency, as applicable.

C.45:1-21.3 Violation of the responsibility to make 911 call, forfeiture of license, authorization to practice.

6. A health care professional licensed or otherwise authorized to practice as a health care professional pursuant to Title 45 of the Revised Statutes who violates the provisions of section 3 of P.L.2003, c.191 (C.30:6D-5.3) shall, in addition to being liable to a civil penalty pursuant to section 4 of P.L.2003, c.191 (C.30:6D-5.4), be subject to revocation of that individual's professional license or other authorization to practice as a health care professional by the appropriate licensing board in the Division of Consumer Affairs in the Department of Law and Public Safety, after appropriate notice and opportunity for a hearing.

C.30:6D-5.6 Rules, regulations.

7. The Commissioner of Human Services, pursuant to the "Administrative Procedure Act," P.L.1968, c.410 (C.52:14B-1 et seq.), shall adopt rules and regulations necessary to effectuate the purposes of this act.

8. This act shall take effect on the 180th day after enactment, but the Commissioner of Human Services may take such anticipatory administrative action in advance as shall be necessary for the implementation of the act.

Approved October 26, 2003.

Division of Developmental Disabilities

HUMAN SERVICES

Identifying Life-Threatening Emergencies

Danielle's Law Training

Training Overview

- Who is Danielle?
- Danielle's Law & Staff Responsibility
- Health-threatening conditions
- Life-threatening emergencies
- End of Life and Hospice
- How to Respond to Life threatening emergencies: When to call 911
- Penalties for Non-compliance of Danielle's Law
- Scenarios
- Interactive Quiz

Who is Danielle?

- 32 year old Danielle Gruskowski lived a full and active life as a member of her family and of the Carteret, NJ community
- She founded the “Carteret Specials” group dedicated to enriching the lives of local children with developmental disabilities



Danielle's Law

Danielle's Law was named in her memory to require 911 to be called for life-threatening emergencies

The law emphasizes the importance of staff calling 911 in life-threatening emergencies for people with intellectual and developmental disabilities or traumatic brain injuries

It’s the Law: Responsibilities

Staff

- Anyone who works with individuals with intellectual and developmental disabilities or traumatic brain injury must call 911 in the event of a life- threatening emergency
- Not calling 911 or delaying the call in a life-threatening emergency is a violation of the law

Employer/Provider

- Ensure staff are trained to identify life- threatening emergencies and know when to call 911
- File an incident report according to Division procedures
- Maintain a record of all 911 calls

Department of Human Services

- Ensure training is provided to service providers
- Maintain records of provider calls to 911
- Review potential violations of Danielle’s Law
  - Situations where a reasonably prudent person did not act in a life-threatening emergency
- Maintain records of violations

Responsibilities & Quality of Care

- You have an important role in caring for individuals with intellectual and developmental disabilities
- Recognizing medical issues and obtaining appropriate care is vital
- Good communication and monitoring of individuals you are assisting provides quality of care
- Your actions can save a life



Identifying Life-Threatening Emergencies

Life-threatening Emergencies are potentially fatal

Need to identify those situations that are life-threatening emergencies

Not all medical situations are life-threatening emergencies

Scenario One

You provide individual supports to Suzie in her own home. One afternoon, as she is walking down the driveway to get her mail, you witness Suzie trip over an uneven patch on the sidewalk and fall to her knees. She cries out in pain and does not immediately stand up. When you reach her to offer assistance, you notice that both her knees are skinned and bleeding. You help her to her feet.

Health-Threatening Conditions

Some medical situations may be health threatening, but not life threatening

- Obtain appropriate medical care for health-threatening conditions, such as calling a doctor
- Always follow established medical protocols for any individual you support, such as checking blood sugar or providing physician prescribed asthma medication
- Report all health conditions to your supervisor and monitor the condition

Scenario Two

Ilene is volunteering at a food bank, where she helps to stack and sort the canned goods into different bins. She does not realize one of the metal bins has a jagged edge, and when she reaches past it, she slices her forearm and it begins to bleed. The cut is long and painful but not deep. You help her to clean the wound and stop the bleeding.

Health-Threatening Conditions

- Examples of Health-Threatening Conditions:
  - Sprained ankle
  - Bleeding controlled with pressure
  - Seizures typical to the person that last under 5 minutes
  - Minor burns/cuts
  - Flu symptoms
- If the health-threatening condition worsens or becomes life threatening, call 911 immediately

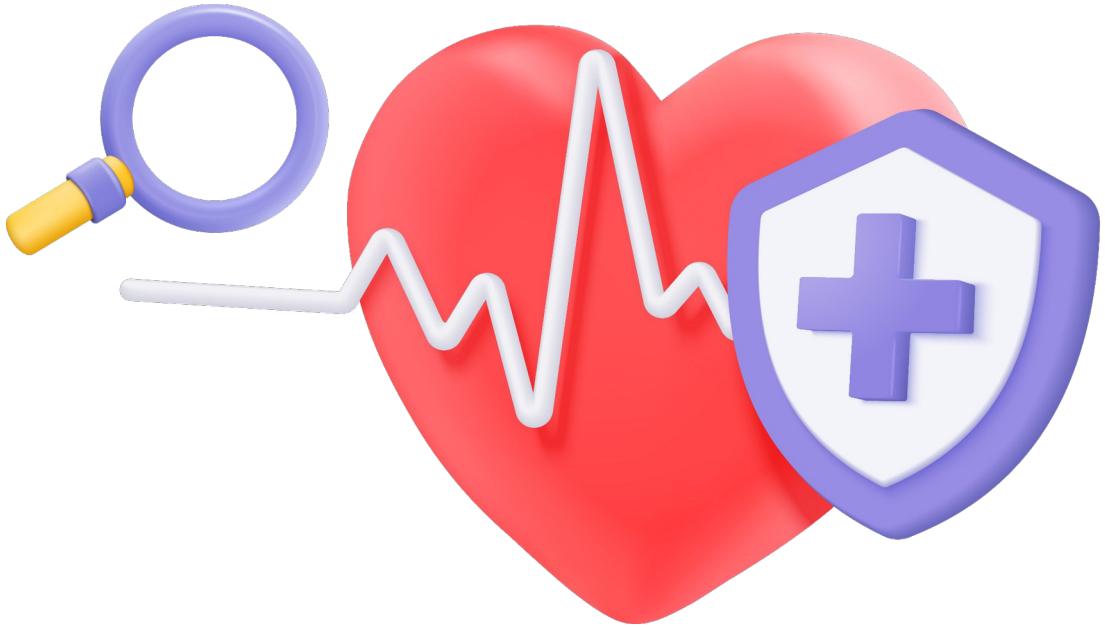


Scenario Three

Will calls you to say he cannot go to work today because he has a sore throat, runny nose, and a cough. He says he did not sleep well last night and is very tired. He is sniffling the whole time you are on the phone and his voice sounds very scratchy. He says he is starting to get the chills and is going back to bed.

Identifying Life-Threatening Emergencies

Life-threatening emergencies are those situations when immediate intervention is necessary to protect a person's life or if serious impairment or dysfunction of a person's body functions or organs/parts may occur



Scenario Four

You are assisting Carmen during her shower. She is seated in a shower chair as you help her dry off. You hear a wheezing sound when she breathes. You lean closer and hear a whistling noise every time she breathes in. You know Carmen has a history of asthma and has asthma medications. After Carmen takes her asthma medications, she is still wheezing strongly.

Identifying Life-Threatening Emergencies

- Examples of life-threatening emergencies include, but are not limited to:
  - Unconsciousness
  - Persistent chest pain or discomfort
  - Not breathing or trouble breathing
  - Severe bleeding
  - Choking
  - Severe, persistent abdominal pain with or without vomiting
  - Serious head injury
  - Shock
  - Some seizures
  - Confusion or disorientation
  - Stroke symptoms

Scenario Five

Frank is finishing his breakfast. He has not shown much appetite this morning, and has only picked at his food, which is unusual for him. Throughout the meal, he has rubbed his left shoulder and arm and made grimacing faces. He nods every time you ask if he feels okay. As he passes you his dishes to put in the sink, you notice he is sweating and pale. He starts to rub his chest and says it feels like someone is standing on his chest.

Recognizing Signs and Symptoms of a Stroke “FAST”

- To identify stroke symptoms, use the acronym “FAST”:
  - **F** = Face: **Ask the person to smile**

    Does one side of their face droop?
  - **A** = Arms: **Ask the person to extend both arms out**

    Does one arm drift downward?
  - **S** = Speech: **Ask the person to repeat a simple sentence**

    Are the words slurred? Can the person repeat the sentence correctly?
  - **T** = Time: **Notate the time signs/symptoms of the stroke first began**

    Call 911 immediately

Scenario Six

One night Ida gets up to use the bathroom and trips over the outstretched legs of her roommate sitting next to her. She falls and doesn't quite catch her fall with her hands. You witnessed her head hit the carpet. She gets up right away, rubbing her head. She is crying a little bit and says that it hurts. When you check her head, she has very faint redness and a little swelling on the left side of her forehead.

Symptoms of a Serious Head Injury

- Examples of serious head injuries include, but are not limited to:
  - Dizziness
  - Headache
  - Blurred vision
  - Nausea/vomiting
  - Confusion or disorientation, slurred speech
  - Lethargy, irritability or excessive sleepiness
  - Bleeding from one or more orifices
  - Numbness or tingling
  - Force of injury caused damage to wall or dents sturdy object
  - Seizure

Scenario Seven

You are assisting several people from the van to their day program to start the day when Carlos slips on some ice and falls backwards, hitting his head on the pavement. He doesn't move right away and seems stunned by the fall. You asked him if he is okay and attempt to sit him up, but he doesn't really respond, although his eyes are open.

When Seizures Are Considered Life-Threatening Emergencies

- Seizures that may be life threatening:
  - First time seizure
  - Lasting longer than 5 minutes
  - Back-to-back (3 or more in a row)
  - Result in serious injury
  - Person is not breathing once seizure stops
  - Occur in someone who is pregnant
  - Occur in someone who has diabetes



Scenario Eight

Jerome is sitting in his chair, working on a puzzle, when his body stiffens, then he begins to breathe deeply before he slumps back in the chair and begins to have jerky movements. You loosen the shirt around his neck and ensure he is breathing. Jerome has a known seizure disorder, so you know to time the seizure as he typically has a seizure that lasts about 2-3 minutes a few times a week. After 3 minutes have passed, his jerky movements stop and he opens his eyes and starts to sit up. He is breathing fine now, but appears tired.

Signs that a Choking Event May be a Life-threatening Emergency

The person:

- Has an airway obstruction
- Has a blue or purplish tint to their face or lips
- Requires back blows and/or abdominal thrusts
- Loses of consciousness-CPR initiated
- Coughing and finger sweeps are ineffective at dislodging the airway blockage
- Presents with respiratory distress or a change of demeanor following the choking event

Scenario Nine

Molly is out to eat with her peers and staff at the mall, when she begins to make gurgling sounds while swallowing. She is coughing and some food comes out of her mouth. She continues to sound like she is gagging and her eyes are watering. She appears to stop breathing and you jump up to use back blows and abdominal thrusts to assist her. It works right away and a piece of food comes out.

Identifying Life-Threatening Emergencies

- Some questions to determine if 911 should be called:
  - Could this condition be potentially fatal?
  - Could the condition get worse and become life threatening if you drove the person to the hospital?
  - Could moving the person on your own cause further injury?
  - Does the person require the skills/equipment of emergency medical personnel?



Danielle's Law and End of Life Orders

- **Danielle's Law requires that 911 is called in the event of a life-threatening emergency, even if the individual has a DNR**
- Orders such as a DNR should be clearly posted in the home so that all staff and caregivers are aware
- When calling 911:
  - Inform the operator of any end of life orders, such as a DNR, DNI, DNH, or POLST
  - Follow the recommendations of the 911 operator
  - Responding emergency personnel will assess and determine appropriate actions

Danielle's Law and Hospice

- For individuals at the end of life receiving **hospice care, regardless of the presence of an end of life order such as a DNR**:
  - When issues arise related to the terminal condition, or if it appears the person has passed away, staff should call the hospice provider
  - **Staff <u>are</u> required to call 911 immediately for life-threatening emergencies <u>unrelated</u> to the terminal condition**
    - For example, if someone had a terminal cancer diagnosis, but experienced emergencies including but not limited to: a choking episode, first time seizure or prolonged seizure of over 5 minutes, or for a serious injury such as a broken bone, injury from a fall, profuse bleeding, etc.
    - In those cases, staff should alert the 911 operator and emergency responders that the individual is on hospice, inform them of any end of life orders, and provide related contact information as needed

Scenario Ten

- June has stage 4 cancer and is receiving hospice care and has a DNR. As per a recent planning meeting, June must be repositioned by staff every three hours as she is no longer able to move on her own. While being repositioned, staff were unaware the bed rail was in the down position, and June fell out of bed, hitting her head on the night stand before landing on the floor. She was bleeding profusely from a deep laceration to her head. June immediately complained of a severe headache. Moments later, she reported changes to her vision and began to have difficulty speaking.

Responding to Life-Threatening Emergencies

- **CHECK** the person to determine if he/she is experiencing a life-threatening emergency
- **CALL 911 If the condition is life threatening**
- **CARE** for the person until help arrives



*You are not required to call 911 in non-life-threatening situations, but appropriate medical care is **always** required*

Life-Threatening Emergencies: Staff Responsibility

- If you identify a life-threatening emergency, CALL 911 immediately
- *Don't* call a supervisor or co-worker first when a life-threatening emergency exists
- If you are not sure if the situation is life threatening, call 911



Scenario Eleven

You are a job coach assisting Harry at his new job in the kitchen at the local restaurant. Harry is in charge of food preparation today, and is chopping salad items at the counter with a sharp knife. Just as you turn to leave, since he is doing well on his own, you hear him cry out in pain. The palm of his hand has a long gaping cut across the whole width of his hand. It is bleeding profusely. You grab a clean towel; wrap it around his hand, and apply pressure. The blood continues to soak right through the thick towel.

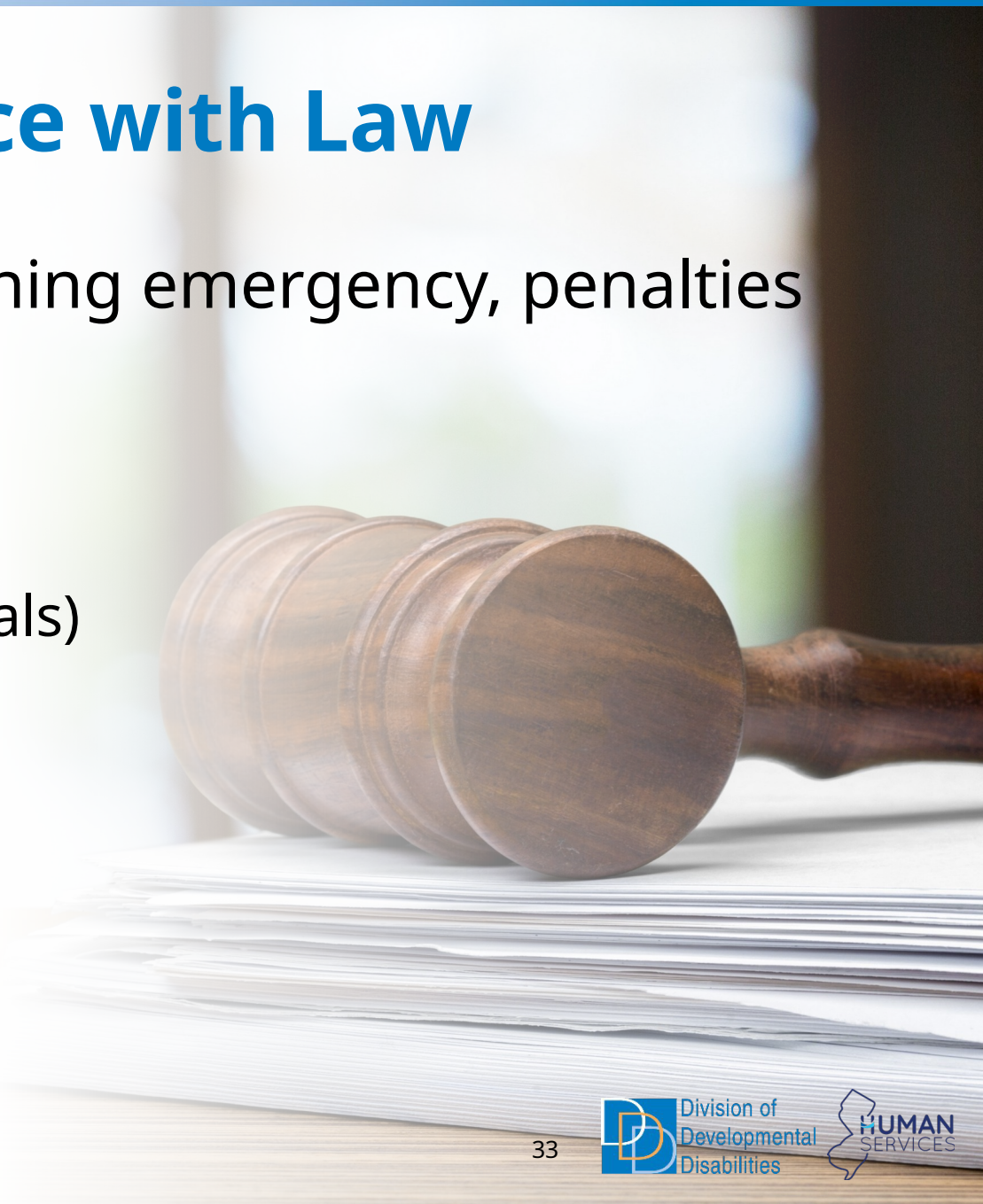
Calling 911

- Tell the 911 Operator:
  - Your name and number
  - Location of incident
  - What happened
  - Condition of person including any special conditions
  - Care being provided
- Don’t hang up until the dispatcher instructs you to do so



Penalties for Non-Compliance with Law

- If you do not call 911 in a life-threatening emergency, penalties may include:
  - Termination from employment
  - Fines up to $25,000
  - Loss of license (for healthcare professionals)



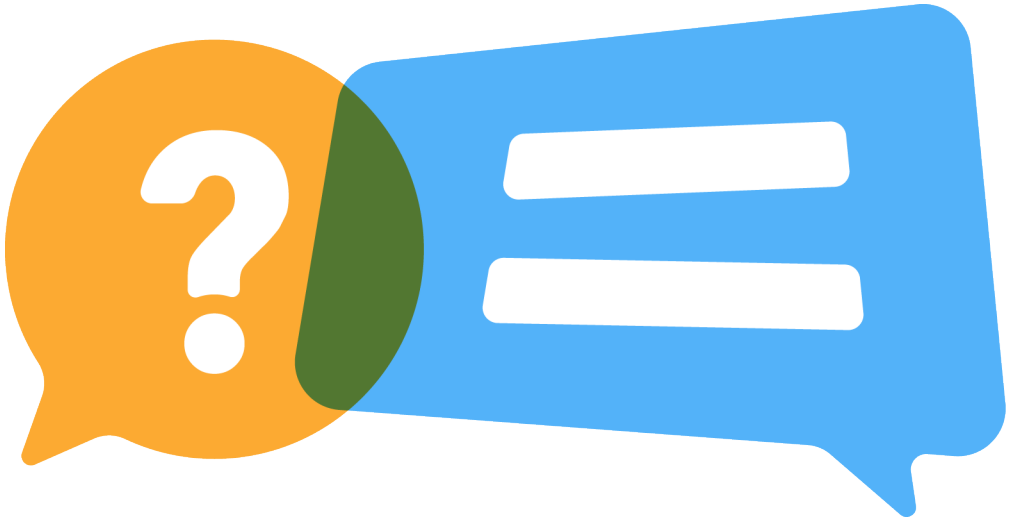
How are Danielle's Law Incidents Reviewed?

- Following OI's Investigation, DDD Risk Management Staff reviews the case
- Criteria:
  - Was the condition a life-threatening emergency?
  - Would a reasonably prudent person have known at the time of the incident that the condition was life threatening?
  - Recommendations are not outcome based
- Recommendation made to Assistant Commissioner for final decision



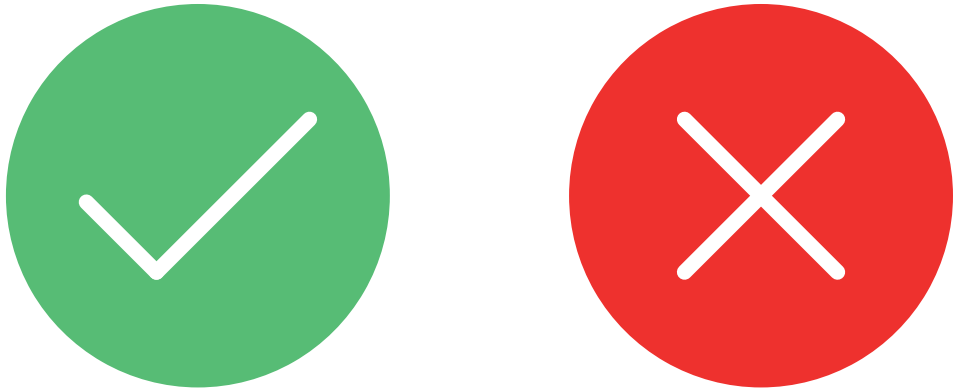
Review & Quiz

- Review of health-threatening conditions
- Review of life-threatening emergencies
- Frequently Asked Questions Quiz



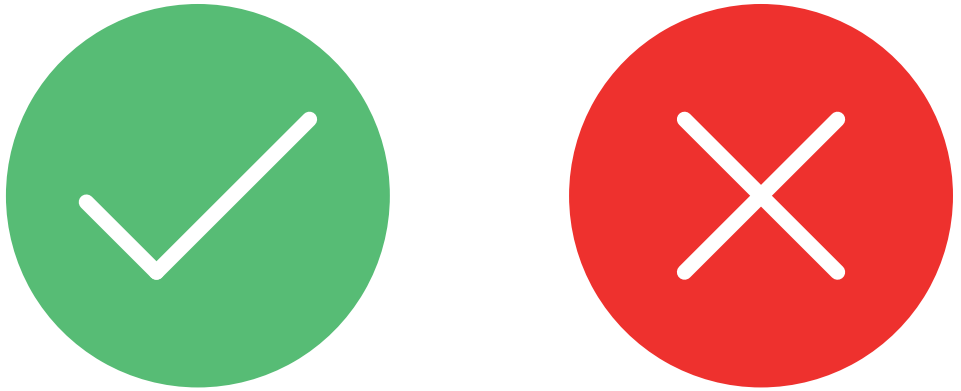
Quiz Question: True or False?

Direct care staff often know the person best, so their input into medical conditions is important.



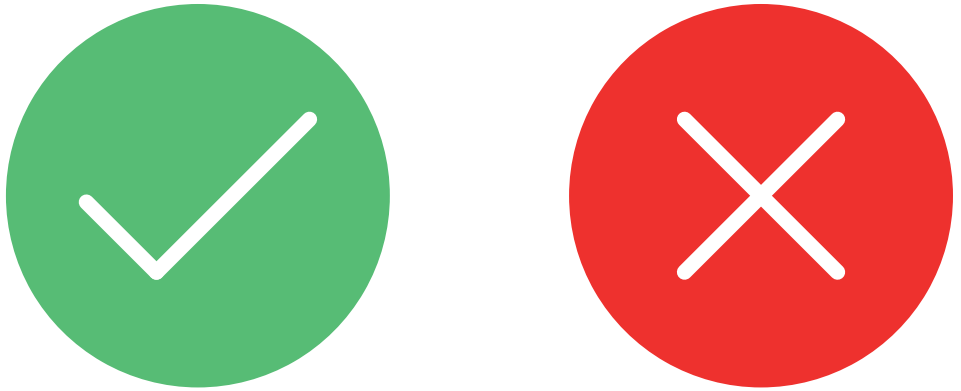
Quiz Question: True or False?

All medical situations and conditions are life-threatening emergencies.



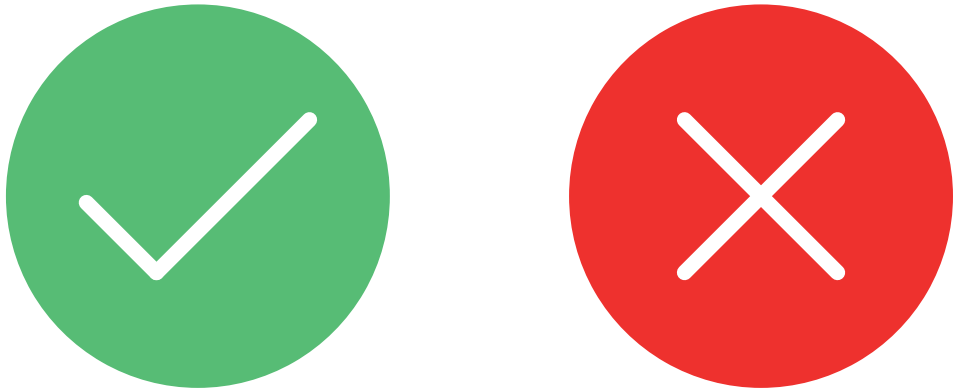
Quiz Question: True or False?

All 911 calls must be for life-threatening emergencies.



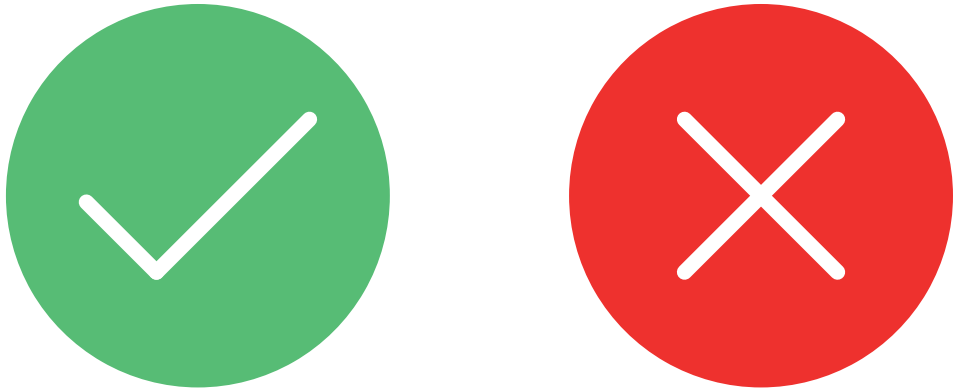
Quiz Question: True or False?

A health-threatening condition may need immediate medical care such as driving someone to the doctor's office or an emergency room.



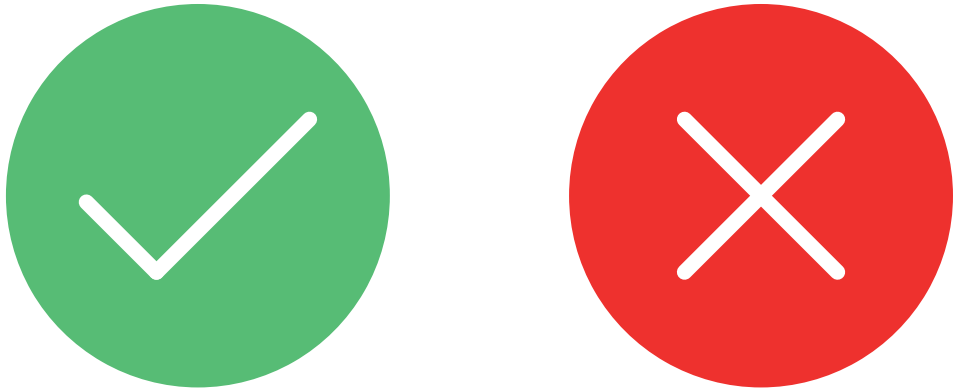
Quiz Question: True or False?

Examples of life-threatening emergencies would be a heart attack or stroke.



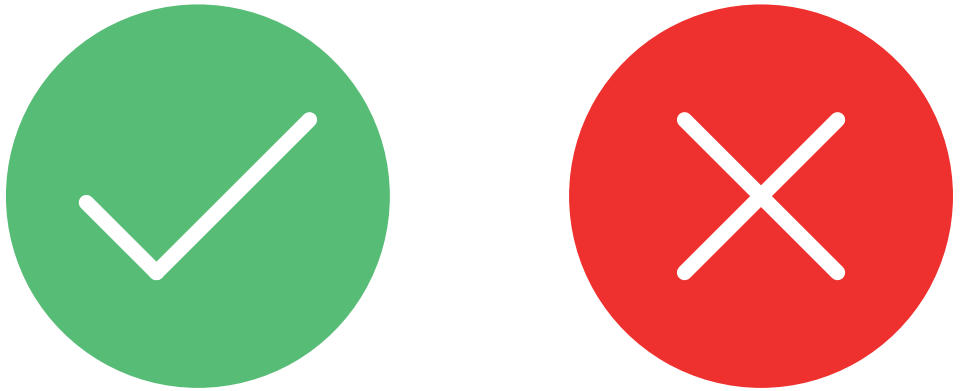
Quiz Question: True or False?

An example of a health-threatening condition would be seizures typical for the person lasting less than 5 minutes.



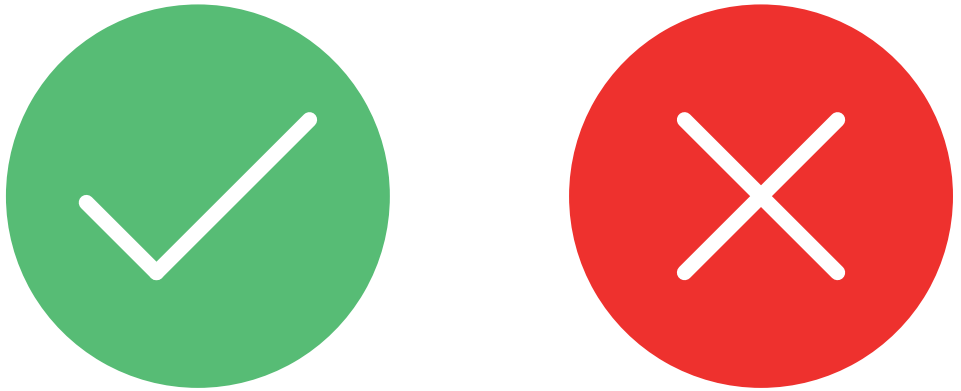
Quiz Question: True or False?

If someone is having trouble breathing, but is conscious and able to talk to you, it would be safe to drive them to the hospital yourself.



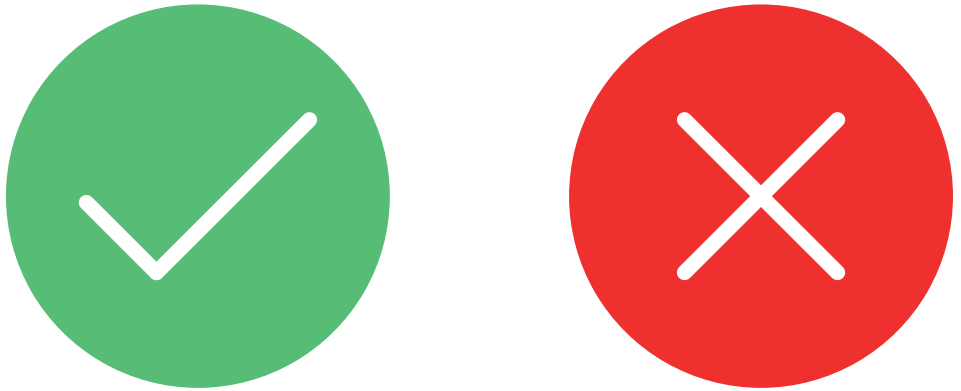
Quiz Question: True or False?

When you identify a life-threatening emergency, you should call 911, provide care and then call your supervisor to report the situation.



Quiz Question: True or False?

Health-threatening conditions are not emergencies, however appropriate and prompt medical care is always required, such as calling a doctor.



Wrap Up

For more information, training materials, and to view this Danielle's Law slide deck, please visit the Division's Danielle's Law Website:



www.nj.gov/humanservices/ddd/providers/staterequirements/danielle/

IDENTIFYING LIFE THREATENING EMERGENCIES

- You must call 9-1-1 in the event of a life-threatening emergency.
- **Ask yourself:**
  - **Could this condition be potentially fatal?**
  - **Could the condition get worse and become life threatening if you drove the person to the hospital on your own?**
  - **Could moving the person on your own cause further injury?**
  - **Does the person require the skills/equipment of emergency medical personnel?**
- Life-threatening emergencies may include:
  - Unconsciousness, unusual confusion/disorientation or losing consciousness
  - Difficulty breathing, not breathing, or breathing in a strange way
  - Having persistent chest pain, discomfort or pressure which persists for more than 3 -5 minutes or that goes away and comes back
  - Severe bleeding from a body part
  - Broken bone that is showing through the skin or severe disfigurement of body part
  - Severe headache with slurred speech
  - Seizures that are not typical or back-to-back (3 in a row)
  - Seizures lasting longer than 5 minutes
  - Seizure resulting in serious injury; seizure in someone who is pregnant; seizure in someone who is diabetic; seizure in someone for the first time
  - Serious injury to head, neck or back
- Call 9-1-1 first ***before*** your supervisor in a life-threatening emergency.
- If you are unsure whether a situation is a life-threatening emergency, call 9-1-1. If the situation is not life-threatening, call your supervisor and provide appropriate care, including obtaining medical attention.

FREQUENTLY ASKED QUESTIONS
ABOUT DANIELLE'S LAW

1. **How can I prepare for an emergency?**

   Attend Training such as CPR/First Aid.

   Get to know each individual you support and keep a fact sheet on each individual that includes:

   - List of all current medications
   - List of medical conditions and medical history
   - Phone number of guardian, next of kin, and physician

2. **Do I need to call 911 if I think that another staff person has called 911?**

   If you are absolutely certain that the 911 call has been made, which means that you see or hear another staff person place the call to 911, then you do not need to call 911 again yourself. However, if you have any doubt, you must call 911.

3. **When a doctor or nurse is available on staff, should I check with them before calling 911?**

   In the event of a life threatening emergency, you must immediately call 911. A doctor or nurse can be notified *after* the 911 call is made so he/she can evaluate and provide medical attention. Even if the situation is *not* life threatening, a doctor or nurse can still evaluate and provide medical attention.

4. **Can I drive the individual to the emergency room?**

   If the individual has a life-threatening emergency, call 911 instead of driving to the emergency department or a doctor's office. If the situation is *not* life threatening, it may be appropriate to drive the person for an evaluation if your supervisor or a medical professional advises you to do so.

5. **When the person, family member, or guardian doesn't want me to call 911, do I still need to call 911?**

   Yes. If the individual has a life threatening emergency, Danielle's Law requires that 911 be called. Handle the refusal of the individual with sensitivity and explain that calling 911 is necessary.

6. **When the individual has a DO NOT RESUSCITATE (DNR) ORDER, do I still need to call 911?**

   Yes. If the individual has a life threatening emergency, even if the individual

has a DO NOT RESUSCITATE ORDER, Danielle's Law requires that 911 is called. Once the call is made, inform the operator of the DNR. The emergency medical professionals who respond will determine what type of medical care should be provided. Have the DNR Order available to show the Emergency Medical Service technician.

7. **What if I don't have access to a phone?**

   In such an instance, yell for help to persons passing by in cars or walking by, and tell them that the individual has a life-threatening emergency and 911 must be called.

8. **Is staff required to call 911 if issues related to the terminal condition of an individual at the end of life arise?**

   Staff does not need to call 911 for an individual receiving end-of-life care when issues arise related to the medical condition; a violation of Danielle's Law will not have occurred. However, staff still must call 911 for life threatening emergencies unrelated to the terminal condition for example for a fall and profuse bleeding or evidence of a serious injury to the head or impairment of a body part, choking, first time seizure or prolonged seizure of more than 5 minutes, etc. During the 911 call, inform the operator if the individual has a Practitioner Order for Life-Sustaining Treatment or a Do Not Resuscitate Order and lastly, you should provide the phone number of the Hospice program once Emergency Medical Services Technicians arrive onsite.

9. **Who will investigate situations in which a 911 call was not made in a life-threatening emergency?**

   The Department of Human Services' Office of Program Integrity and Accountability (OPIA) will investigate alleged violations. The Division's Risk Management staff will evaluate the investigative evidence and will present its findings to the Assistant Commissioner who will determine whether or not a violation has occurred.

10. **Will a community care residence provider, developmental center employee, or agency staff member have the right to appeal a licensing action or fine imposed as a result of violating Danielle's Law?**

    Yes. The letter that a violator of Danielle's Law receives will inform him or her of the fine and the appeal process.

DANIELLE'S LAW IN-SERVICE:
IS THIS A LIFE-THREATENING EMERGENCY?

DIRECTIONS:

In a small group, discuss the following scenarios to determine if the situation is a life-threatening emergency requiring a 911 call. Decide how you would handle the situation. Choose a group leader as spokesperson in the event you are asked to share your answers with the larger group.

SCENARIO #1: You provide individual supports to Suzie in her own home. One afternoon, as she is walking down the driveway to get her mail, you witness Suzie trip over an uneven patch on the sidewalk and fall to her knees. She cries out in pain and does not immediately stand up. When you reach her to offer assistance, you notice that both her knees are skinned and bleeding. You help her to her feet.

SCENARIO #2: Ilene is volunteering at a food bank where she helps to stack and sort the canned goods into different bins. She does not realize one of the metal bins has a jagged edge and when she reaches past it, she slices her forearm and it begins to bleed. The cut is long and painful, but not deep. You help her to clean the wound and stop the bleeding.

SCENARIO #3: Will calls you to say he cannot go to work today because he has a sore throat, a runny nose, and a cough. He says he did not sleep well last night and is very tired. He is sniffling the whole time you are on the phone and his voice sounds very scratchy. He says he is starting to get the chills and is going back to bed.

SCENARIO #4: You are assisting Carmen during her shower. She is seated in a shower chair and as you help her dry off, you hear a wheezing sound when she breathes. You lean closer and hear a whistling noise every time she breathes in. You know Carmen has a history of asthma and has asthma medications. After Carmen takes her asthma medications, she is still wheezing strongly.

SCENARIO #5: Frank is finishing his breakfast. He has not shown much appetite this morning, and has only picked at his food, which is unusual for him. Throughout the meal, he has rubbed at his left shoulder and arm and made grimacing faces. He nods every time you ask if he feels okay. As he passes you his dishes to put in the sink, you notice he is sweating and pale. He starts to rub his chest and says it feels like someone is standing on his chest.

SCENARIO #6: One night, Ida gets up to use the bathroom and trips over the outstretched legs of her roommate sitting next to her. She falls and doesn't quite catch her fall with her hands. You witness her head hit the carpet. She gets up right away, rubbing her head. She is crying a little bit and says that it hurts. When you check her head, she has very faint redness and a little swelling on the left side of her forehead.

SCENARIO #7: You are assisting several people from the van to their day program to start the day when Carlos slips on some ice and falls backwards, hitting his head on the pavement. He doesn't move right away and seems stunned by the fall. You ask him if he is okay and attempt to sit him up, but he doesn't really respond although his eyes are open.

SCENARIO #8: Jerome is sitting in his chair, working on a puzzle, when his body stiffens, then he begins to breathe deeply before he becomes unconscious. He slumps back in the chair and begins to have jerky movements. You loosen the shirt around his neck and ensure he is breathing. Jerome has a known seizure disorder, so you know to time the seizure as he typically has seizures that last about 2-3 minutes a few times a week. After 3 minutes have passed, the jerky movements stop and he opens his eyes and starts to sit up. He is breathing fine now but appears tired.

SCENARIO #9: Molly is out to eat with her friends at the mall when she begins to make gurgling sounds while swallowing. She begins to cough and some food comes out of her mouth. She continues to sound like she is gagging and her eyes are watering. She appears to stop breathing and you jump up to use back blows and abdominal thrusts to assist her. It works right away and a piece of food comes out.

SCENARIO #10: June has stage 4 cancer and is receiving hospice care and has a DNR. As per a recent planning meeting, June must be repositioned by staff every three hours as she is no longer able to move on her own. While being repositioned, staff were unaware the bed rail was in the down position, and June fell out of bed, hitting her head on the night stand before landing on the floor. She was bleeding profusely from a deep laceration to her head. June immediately complained of a severe headache. Moments later, she reported changes to her vision and began to have difficulty speaking.

SCENARIO #11: You are a job coach assisting Harry at his new job in the kitchen at the local restaurant. Harry is in charge of food preparation today and is chopping salad items at the counter with a sharp knife. Just as you turn to leave since he is doing well on his own, you hear him cry out in pain. The palm of his hand has a long gaping cut across the whole width of his hand. It is bleeding profusely. You grab a clean towel, wrap it around his hand, and apply pressure. The blood continues to soak right through the thick towel.